VictoryAI

The AI Recovery Operating System for Opioid Use Disorder

SERIES A · $12M · 2026

$42B
MARKET

5.7M
PATIENTS

$84M
ARR BY 2030

89%
GROSS MARGIN

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America's deadliest disease still runs on paper.

$1.5 trillion a year. Zero clinical visibility between visits.

$1.5T
Annual U.S. cost
CDC · Joint Econ. Committee

5.7M
Americans with OUD
Only 22% receive any MAT

60%
Relapse in 30 days
After leaving treatment

220
Overdose deaths/day
80,000+ Americans/year

There is no Epic for addiction. There is no Livongo for OUD.
The category is wide open — and we're building it.

Five tailwinds. One window.

The Pear Therapeutics bankruptcy proves the single-FDA-reimbursement model fails. We built three.

1	**Reimbursement Unlocked**	CMS now pays $80–200/pt/month for RTM 98980, CCM 99490, G2214, TCM 99495/96, and E/M uplift — codes that require between-visit data we capture.
2	**Pear Failed — The Market Didn't**	Pear had FDA clearance and still filed Ch.11 (2023). One revenue path = one point of failure. VictoryAI has three. The OUD digital market is $9.6B and accelerating.
3	**Payers Demand Measurable Outcomes**	Medicaid SUD contracts moving to value-based care. No outcomes data = no contract renewal. DHCS DMC-ODS now requires documented MAT adherence metrics.
4	**$50B in Opioid Settlement Funds**	50 states deploying 2024–2030, explicitly earmarked for OUD treatment innovation. Grants available without payer approval. CA alone: $2B+ allocated.
5	**AI Crossed the Threshold**	GPT-class models deliver 24/7 trauma-informed CBT at $0.04/session. Relapse prediction 5–7 days pre-crisis is deployable today. The infrastructure is here.

Between-visit data unlocks $80–200 per patient per month.

$80–200

/patient/month · CMS CY2024

CODE	DESCRIPTION + BILLING PREREQUISITE	CA	NY	MA	RANGE /PT/MO
98980	**RTM — Remote Therapeutic Monitoring (first 20 min/mo)** ≥16 engagement days + 1 interactive clinician comm.	$58	$65	$68	$45–90
99490	**CCM — Chronic Care Management (first 20 min/mo)** ≥20 min documented care coordination · 75% of OUD pts qualify	$62	$68	$72	$44–72
G2214	**OUD Continuation — ≥11 consecutive MAT days** Daily MAT confirmations → timestamped consecutive-day log	$48	$53	$56	$30–56
99495	**TCM — Post-Discharge Transitional Care (14-day)** 2-day post-discharge contact auto-captured via app check-in	$201	$220	$231	per event
99213+	**E/M Visit Complexity Uplift** Between-visit data supports higher complexity visit documentation	+$23	+$28	+$30	+$19–40

Worked example: 100-patient clinic · 65% engagement · RTM + CCM + G2214 + E/M uplift = ~$12,000/month in reimbursement documentation · Platform cost: $14.99/pt/mo = $1,499/month · ROI: 8x

One platform. Three products. Three ways to win.

Patient phone → clinician dashboard → payer claim. Same AI infrastructure. Three independent margin lines.

PATIENT

Sage AI Companion

$19.99 / month

Direct B2C — App Store

Consumer pays direct. No clinic required.

- 24/7 OUD-trained AI coach (Sage)
- MAT tracker — Suboxone / Methadone / Naltrexone
- COWS withdrawal scoring
- 50-lesson CBT recovery workbook
- Family Reward Board + gamification
- Crisis alert + Narcan locator

CLINICIAN

Outcomes Dashboard

$5,000 / facility / yr

B2B SaaS — direct to clinic

Annual contract. Low friction. Pilot → paid.

- Real-time patient risk scores
- AI relapse prediction (5–7 days early)
- MAT adherence + engagement analytics
- Drug screen & group attendance logs
- COWS trend + cohort outcomes
- DHCS / DMC-ODS reporting export

REVENUE

Billing Intelligence

$14.99 / patient / mo

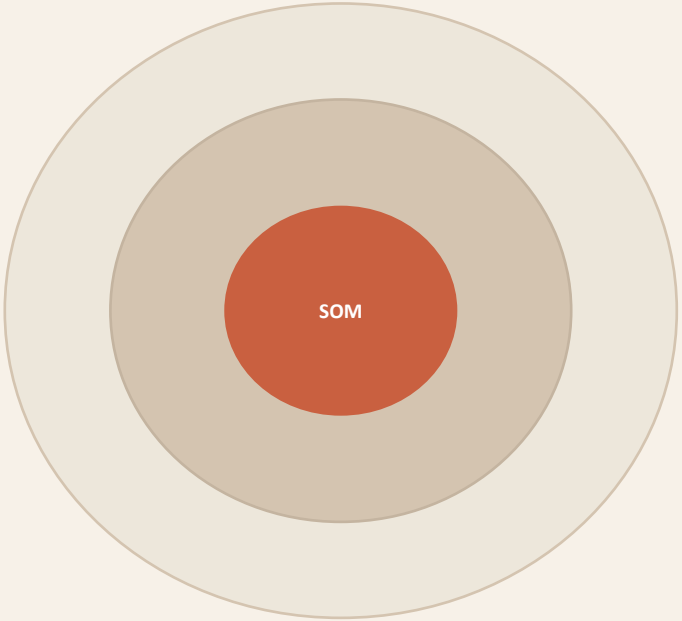
Enterprise — clinic-deployed bulk tier

$14.99 ≠ discount. Clinic deploys at scale; you capture $80–200/pt/mo for them.

- RTM 98977/98980 — $35–89/pt/mo
- CCM 99490 / PCM 99424 — $62–82/pt/mo
- G2213/G2214 OUD codes — $48–162 episode
- TCM 99495/99496 — $201–273 per event
- E/M uplift — +$23–40/visit
- Kipu EHR auto-sync · prereq flags

A $42B market hiding inside a $1.5T crisis.

Growing 11% CAGR. Less than 1% digitized today.



TAM **$42B**

U.S. behavioral health & SUD treatment

11.4% CAGR through 2034

SAM **$9.6B**

Digital OUD + MAT technology platforms

Fastest-growing SUD subsegment

SOM **$460M**

CA Medi-Cal beachhead → 5-state expansion

240 OUD clinics in CA alone today

$50B opioid settlement funds · CMS digital codes live · 75% of OUD patients still untreated · DHCS DMC-ODS outcomes mandate 2025

Three revenue streams. One patient. ROI is clear.

Land with the patient → expand to the clinic → monetize the payer claim.

Pricing clarity: $19.99/mo = direct B2C consumer price. $14.99/pt/mo = enterprise bulk rate when clinic deploys to patients at scale. Lower unit price; higher volume + $5K facility fee on top. The platform captures $80–200/pt/mo in reimbursement — the $14.99 cost is 7–13% of value delivered.

B2C

Patient App

$19.99 / month

92% gross

Freemium → premium. Family rewards drive viral loop. App Store + Google Play.

$360
LTV

$18
CAC

< 1 mo
Payback

B2B

Provider SaaS

$5,000 / facility / yr

88% gross

Annual contract. Outcomes dashboard + AI relapse engine. Pilot → paid → multi-site.

$35K
LTV

$2K
CAC

5 mo
Payback

ENTERPRISE

Clinic App Bundle

$14.99 / patient / mo

94% gross

Clinic pays $5K site fee + $14.99/pt. Platform ROI: 8x on reimbursement capture alone.

$48K
LTV

$4.5K
CAC

9 mo
Payback

Everyone solves a slice. We own the loop.

Pear's failure is our proof point: one FDA-dependent revenue path = one point of failure. Three paths = resilience.

COMPETITOR	OUD-Specific	AI Companion	Clinician Dash	Billing Intel	Family Gamif.	Between-Visit
Pear / Reset-O (defunct)	✓	—	—	—	—	—
DynamiCare	✓	—	partial	—	—	—
WEconnect	partial	—	✓	—	—	—
Bicycle Health	✓	—	internal	—	—	—
Workit Health	✓	—	internal	—	—	—
VictoryAI	✓	✓	✓	✓	✓	✓

WHY WE WIN

- Only full-stack OUD platform — patient + clinician + payer in one loop. Three revenue paths, not one FDA bet.
- Proprietary AI: relapse prediction 5–7 days pre-crisis · between-visit data generates $80–200/pt/mo reimbursement.
- 42 CFR Part 2 compliance moat: every competitor building this faces 18 months of SUD data segregation work.

Pilots signed. Product live. Pipeline loaded.

Three named design partners. Channel live. First paying conversions targeted Q3 2026.

3
OUD networks in pilot LOI
90-day design-partner agreements

240+
Medi-Cal clinics reachable
Via CA beachhead pipeline

$2.4M
Pipeline ARR from signed LOIs
3 converting to paid Q3 2026

70%
CA SUD market via Kipu channel
Non-exclusive; direct EHR pipeline in parallel

KEY DESIGN PARTNERS & CHANNEL

DESIGN PARTNER

BayMark Health Services

Largest U.S. OUD network · 400+ clinics · 90-day design-partner pilot signed · co-authored IRB outcomes study (Advarra/WCG) · Not yet a paying customer — first commercial conversion target: Q4 2026

ACTIVE PILOT

California Medi-Cal Network

6-facility MAT/IOP/PHP pilot · independent academic outcomes study underway · DHCS DMC-ODS aligned reporting workflow validated

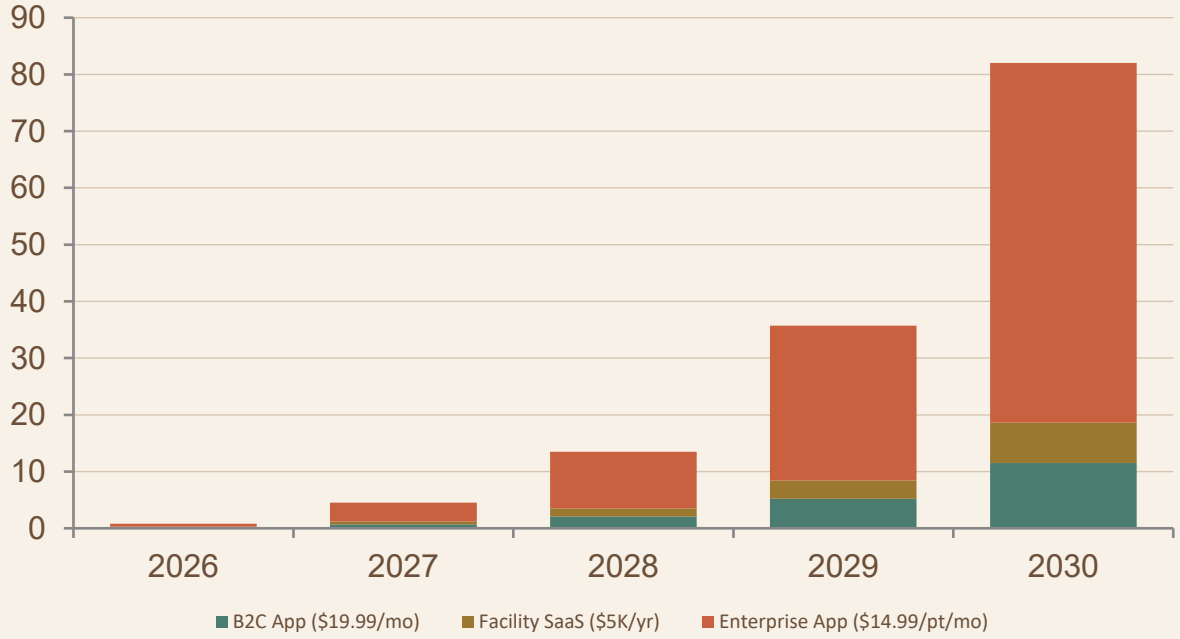
INTEGRATION LIVE

Kipu EHR — Strategic Channel Partner

Integration live · 70% of CA SUD market · Non-exclusive agreement · Building parallel direct integrations with Credible, Netsmart, and Welligent to reduce channel concentration risk

$0.8M → $82M ARR in five years.

Revenue model rebuilt on corrected pricing: $19.99 B2C · $5K facility · $14.99/pt enterprise. T2D3 growth profile.



KEY METRICS · 2030	
Total ARR	$82M
Gross Margin	89%
Facilities (B2B)	1,400
Enterprise Pts	38,000
B2C Subscribers	48,000
Net Rev Retention	132%
LTV:CAC (B2B)	11:1
EBITDA Margin	24%

2026 $0.8M · 2027 $4.5M · 2028 $13.5M · 2029 $35.7M · 2030 $82.0M

Note: Prior deck used $25/pt/mo B2B pricing. Revised to $14.99/pt/mo enterprise + $5K facility fee. Revenue curve reflects corrected unit economics.

Raising $12M Series A

to scale from beachhead to category leader.

USE OF FUNDS

45% **Engineering & Product**
AI relapse engine · EHR integrations · mobile + dashboard

25% **Sales & GTM**
Enterprise AEs · clinical liaisons · payer partnerships

15% **Clinical & Outcomes**
RCT-grade study · KOL advisory · FDA pathway prep

10% **Compliance & Infra**
HIPAA · 42 CFR Part 2 · SOC 2 Type II

5% **Reserve**
Working capital and contingency

18-MONTH MILESTONES

- $8M ARR run-rate exiting Month 18
- 150+ paying OUD clinics across 5 states
- 20,000 B2C paid subscribers at $19.99/mo
- Peer-reviewed clinical outcomes study published
- 2 commercial payer value-based contracts signed
- Series B-ready · $25M ARR trajectory

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Founder-led. Mission-built.

AP

Alpa Patel — Founder & CEO

16+ years scaling tech and growth-stage businesses. B.S. (Honors) Business Information Systems, UT Arlington. Built and sold previous startup. Founded VictoryAI to close the OUD treatment gap with AI and real-time clinical analytics.

Co-Founder & Chief Product & Clinical Officer

JC

John J. Curtin, PhD

Co-Founder, Clinical Officer — Addiction Science

Professor, University of Wisconsin–Madison. Leading OUD relapse prediction researcher. Advisory engagement signed.

BD

Board-Cert Addiction MD — In Negotiation

Medical Advisory · ASAM / AAAP connected

KOL with active Medicaid MCO relationships in CA and OH. Name disclosed under NDA at meeting.

HT

Health-Tech Operator — In Formation

Series B+ exit · digital health scale

Former executive at a digital health company with $200M+ ARR. Reference available on request.

Every person in recovery deserves a coach in their pocket and a clinician who sees them clearly.

OUD today → AUD → Stimulant → the full $42B SUD market. VictoryAI is the OS for addiction recovery.